

1. Administrative & Study Identification

Full Study Title: A Phase 2, Multicenter, Open-label Study to Evaluate the Efficacy and Safety of Trastuzumab Deruxtecan (T-DXd, DS-8201a) for the Treatment of Selected HER2 Expressing Tumors **Short Title/Acronym:** A Phase 2 Study of T-DXd in Patients With Selected HER2 Expressing Tumors / DESTINY-PanTumor02 (DPT02) **Protocol Number:** D967VC00001 **NCT Number:** NCT04482309 **Posting ID:** 383008196283436816 **Sponsor Name / Company:** AstraZeneca **Investigational Product:** Trastuzumab deruxtecan (T-DXd, Enhertu) | **ATC Code:** L01FD04 **Phase of Development:** Phase 2 **Trial Status:** Recruiting (Currently recruiting participants) **Medical Monitor:** AstraZeneca Clinical Operations Lead

2. Study Synopsis & Rationale

2.1 Study Objective Primary Objective: To evaluate the efficacy of Trastuzumab deruxtecan (T-DXd) by assessing the Confirmed Objective Response Rate (ORR) per RECIST 1.1 as assessed by the Investigator. **Secondary Objectives:** To evaluate the Duration of Response (DOR), Progression-Free Survival (PFS), Disease Control Rate (DCR), Overall Survival (OS), and overall safety and tolerability.

2.2 Background & Rationale To address the critical unmet medical need in patients with locally advanced, unresectable, or metastatic HER2-expressing solid tumors who have progressed following prior systemic treatments and lack satisfactory alternative options. This study tests the clinical activity and risk-benefit profile of T-DXd across multiple difficult-to-treat tumor cohorts, structured in two parts: * **Part 1:** Enrolling 7 cohorts including urothelial bladder cancer, biliary tract cancer, cervical cancer, endometrial cancer, epithelial ovarian cancer, pancreatic cancer, and rare tumors. * **Part 2:** Enrolling 5 cohorts (A to E) focusing on HER2 expressing/amplified solid tumors (excluding breast, gastric, colorectal), accommodating specific expression levels (e.g., IHC 3+, IHC 2+/ISH+, and IHC 2+ or 1+ for endometrial, ovarian, and cervical cancers).

3. Study Design & Methodology

3.1 Design Framework Study Type: Interventional **Control Type:** Single-arm (Multi-cohort) **Blinding:** Open-label **Randomization:** N/A

3.2 Planned Enrollment & Duration Target \$N\$: 468 (Targeted Enrollment) **Trial Status:** Recruiting (Currently recruiting)

participants) **Number of Sites:** Multicenter (Global) **Estimated Study Start:** 18-Aug-2020 **Estimated Primary Completion:** 30-Jul-2027

4. Subject Selection (Eligibility Criteria)

4.1 Inclusion Criteria 1. Locally advanced, unresectable, or metastatic disease based on most recent imaging, with measurable target disease assessed by the Investigator based on RECIST 1.1. 2. Disease progression following prior treatment or no satisfactory alternative treatment option (prior HER2 targeting therapy is permitted). 3. Protocol-defined adequate organ function (cardiac, renal, hepatic). 4. HER2 expression scored using current ASCO/CAP guidelines for scoring HER2 for gastric cancer: * **Part 1:** IHC 3+ or IHC 2+ by local or central assessment in Biliary tract, Bladder, Cervical, Endometrial, Epithelial ovarian, Pancreatic, or Rare tumors. * **Part 2:** IHC and ISH results by central assessment as pre-defined for each cohort (Cohort A: IHC 3+ any solid tumor; Cohort B: IHC 2+/ISH+ any solid tumor; Cohort C: IHC 2+ or 1+ endometrial; Cohort D: IHC 2+ or 1+ ovarian; Cohort E: IHC 2+ or 1+ cervical). Non-small cell lung cancer (NSCLC) may be included in Cohorts A and B.

4.2 Exclusion Criteria 1. History of non-infectious pneumonitis/ILD that required steroids, current ILD, or suspected ILD that cannot be ruled out by imaging at screening; lung-specific intercurrent clinically significant severe illnesses. 2. Uncontrolled infection requiring intravenous (IV) antibiotics, antivirals, or antifungals. 3. Pleural effusion, ascites, or pericardial effusion that requires drainage, peritoneal shunt, or Cell-free and Concentrated Ascites Reinfusion Therapy (CART). 4. Known Somatic DNA mutation of HER2 (ERBB2) without tumoral HER2 protein expression. 5. Primary diagnosis of adenocarcinoma of the breast, colon/rectum, or gastric body/gastro-esophageal junction in all parts. NSCLC is excluded in Part 1, but permitted in Part 2. 6. Medical conditions that may interfere with the subject's participation in the study.

5. Intervention & Treatment Plan

5.1 Investigational Regimen Drug Name: Trastuzumab deruxtecan (ATC Code: L01FD04) **Dosage/Frequency:** 5.4 mg/kg given once every 3 weeks (21-day cycle). **Route of Administration:** Intravenous (IV) infusion. **Duration of Treatment:** Administered continuously until disease progression, death, withdrawal of consent, or unacceptable toxicity.

5.2 Concomitant & Prohibited Therapy Permitted: Standard supportive therapies for management of disease-related symptoms and adverse events (e.g., anti-emetics for nausea). **Prohibited:**

Concomitant use of other investigational therapies or anticancer agents during the study treatment period.

6. Study Timeline & Visit Schedule

| Visit ID | Window (Days) | Key Activities/Procedures | | :---
| :--- | :--- | | **Screening** | Day -28 to 0 | Informed Consent, Medical History, Tumor Imaging, HER2 Status Confirmation | | **Baseline** | Day 0 | First IV Infusion (Cycle 1, Day 1), Baseline Labs, Vitals | | **Interim** | Every 21 Days | Treatment Cycles: Safety Labs, AE/SAE monitoring, T-DXd Infusion | | **Tumor Assessment** | Weeks 6-9 | Radiographic imaging for ORR/PFS per RECIST 1.1 | | **End of Study** | Follow-up | Final Vital Signs, Safety Follow-Up, Survival Status Monitoring |

7. Outcome Measures (Endpoints)

7.1 Primary Endpoint Objective Response Rate (ORR): Confirmed ORR per RECIST 1.1 is the percentage of patients with Complete Response or Partial Response that is subsequently confirmed. Assessed by the Investigator.

7.2 Secondary & Exploratory Endpoints * Duration of response (DoR). * Disease control rate (DCR). * Progression free survival (PFS). * Overall Survival (OS) and safety/tolerability graded according to NCI CTCAE.

8. Safety & Adverse Event Reporting

Adverse Event (AE) Monitoring: Routine safety monitoring at each cycle, with specific attention to neutropenia, anemia, and fatigue. **Serious Adverse Events (SAEs):** A critical focus is on the monitoring, reporting, and adjudication of drug-related interstitial lung disease (ILD)/pneumonitis. **Data Monitoring Committee (DMC):** Safety profiles are routinely reviewed by the Sponsor and associated regulatory/safety boards.

9. Statistical Analysis Plan (SAP) Summary

Analysis Populations: Intent-to-Treat (ITT) sets divided by specific tumor cohorts and by HER2 IHC expression levels. **Sample Size Justification:** Study designed to detect a clinically meaningful improvement in ORR in heavily pre-treated populations compared to historical standard-of-care. Total targeted enrollment is 468 participants. **Handling of Missing Data:** Time-to-event outcomes (PFS, OS, DoR) utilize Kaplan-Meier estimates and are right-censored if patients are lost to follow-up or have not experienced an event by data cutoff.

10. Quality Assurance & Regulatory Compliance

GCP Compliance: This study will be conducted in accordance with Good Clinical Practice (GCP) and the Declaration of Helsinki.

Monitoring Plan: Routine onsite and remote monitoring for protocol adherence. **Audit Trail:** Independent Central Review (ICR) for RECIST 1.1 radiographic assessments and central assessment for HER2 IHC/ISH scoring to ensure unbiased outcome measurement.

11. Study Identifiers & Governance

Primary ID: D967VC00001 **Registry Number (NCT):** NCT04482309

Posting ID: 383008196283436816 **Sponsor/Company:** AstraZeneca **Study Title:** DESTINY-PanTumor02 **Official Regulatory Title:** A Phase 2, Multicenter, Open-label Study to Evaluate the Efficacy and Safety of Trastuzumab Deruxtecan (T-DXd, DS-8201a) for the Treatment of Selected HER2 Expressing Tumors

12. Clinical Framework (Oncology Specific)

Primary Condition: HER2-positive cancer / HER2-expressing solid tumors **Disease Areas:** Bladder, Biliary Tract, Cervical, Endometrial, Ovarian, Pancreatic Cancer, Rare Tumors, and any other solid tumor type excluding breast, gastric, colorectal cancer. **Diagnosis Threshold:** Assessed via ASCO/CAP guidelines.

Part 1: IHC 3+ or 2+. Part 2: Pre-defined by cohort, including IHC 2+/ISH+ or IHC 1+ dependent on tumor type. **Medical**

Methodology: Targeted Antibody-Drug Conjugate (ADC) Therapy (ATC: L01FD04) **Optimization Target:** Objective tumor regression and improved survival

13. Study Procedures & Methodology

Management Pathway: Tumor biopsy/archival tissue screening for HER2 expression $\rightarrow$ Enrollment into tumor-specific cohort (Part 1 or Part 2) $\rightarrow$ Continuous 21-day cycles of therapy. **Education Protocol (HCP):** Stringent guidelines for the prompt identification and management of Interstitial Lung Disease (ILD). **Education Protocol (Patient):** Counseling on the risk of respiratory symptoms and embryo-fetal toxicity. **Quality Audit Mechanism:** Central laboratory confirmation of HER2 status (mandatory for Part 2) and centralized imaging readouts.

14. Observation & Follow-Up Schedule

Total Duration: Follow-up continues until disease progression or death; OS assessed up to study end. **Onsite Visit Cadence:** Every 3

weeks (Day 1 of each 21-day cycle). **Remote Monitoring Frequency:** Routine adverse event monitoring between cycles. **Checklist Review Interval:** Radiographic tumor response scans typically assessed every 6-9 weeks.

15. Sign-off & Approval

Role	Name	Signature	Date	:--	:--	:--	:--
Principal Investigator	[Name]	_____	[DD-MMM-YYYY]				
Clinical Operations Lead	[Name]	_____	[DD-MMM-YYYY]				Lead
Statistician	[Name]	_____	[DD-MMM-YYYY]				